Hypnobirthing

A Complete Guide for You and Your Birth Partner

Six sessions · Starting from 28 weeks · My Instant Midwife

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How to Use This Course

This course is designed to be worked through one session at a time, starting ideally around 28 weeks of pregnancy. Each session takes around 30–45 minutes to read, and comes with a workbook section and homework to complete before the next session.

You don't need to rush. Take your time with each session. Read it together with your birth partner if you can — a lot of the exercises work best when you do them as a pair.

By the time you finish, you'll have a full toolkit for birth — breathing techniques you've actually practised, a birth environment you've thought through, a partner who knows exactly how to support you, and a much calmer head than you started with.

A note on language

Throughout this course we use the word 'you' to mean the person who is pregnant. We refer to your birth partner as 'your partner' — but this can be anyone you choose: your husband, wife, girlfriend, mum, best friend, or a hired doula. Whoever is in your corner, this course is for them too.

> **Research note**
>
> *Hypnobirthing is backed by a growing body of evidence. A 2015 Cochrane review of relaxation-based techniques in labour found that women who used relaxation methods reported lower levels of pain, less need for pharmacological pain relief, and higher satisfaction with their birth experience. The techniques in this course are grounded in that evidence.*

Session 1 — What Is Hypnobirthing?

Best completed around 28–30 weeks

Let's start with what it isn't

Hypnobirthing is not being hypnotised by a stranger while whale music plays. It is not a guarantee of a pain-free birth. It is not a philosophy that tells you interventions are bad. And it is definitely not something only certain types of people do.

Honestly? The word 'hypnobirthing' puts a lot of people off — and that's a shame. Because what it actually is, is practical, evidence-based preparation for one of the biggest physical events of your life.

Think of it like this: if you were running a marathon, you wouldn't just show up on the day and hope for the best. You'd train. You'd learn to breathe efficiently. You'd prepare your body and your mind. Hypnobirthing is training for birth.

So what actually is it?

Hypnobirthing is a set of techniques — breathing, relaxation, visualisation, and education — that help you approach labour with less fear and more control. It works by training your mind to stay calm, which in turn allows your body to work more efficiently.

The core idea is simple: a calm mind creates a calm body, and a calm body gives birth more easily. This isn't wishful thinking — it's basic physiology.

A bit of science — for the curious (and for the partners)

Here is the bit that tends to make sense to people who like facts.

When you feel scared or threatened, your body releases adrenaline — the fight-or-flight hormone. This is brilliant if you need to run from a lion. It's less brilliant during labour, because adrenaline causes your uterine muscles to tighten and compete against each other. That makes contractions feel more painful and slows labour down.

On the other hand, when you feel safe and relaxed, your body releases oxytocin — sometimes called the love hormone. Oxytocin is what actually drives labour contractions. The calmer you are, the more efficiently it flows. More oxytocin = more effective contractions = shorter, more manageable labour.

> **The Fear-Tension-Pain cycle**
>
> *In 1933, obstetrician Grantly Dick-Read described what he called the Fear-Tension-Pain cycle. Fear creates tension in the body. Tension makes pain feel more intense. More pain increases fear. And the cycle continues.*
>
> *Hypnobirthing breaks that cycle at the source — by reducing fear before labour even begins. When there's less fear, there's less tension. Less tension means pain is more manageable. It really is that simple.*

The role of your mind in birth

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Your brain doesn't fully distinguish between a real experience and a vividly imagined one. This is why horror films make your heart race even though you know they're not real. It's also why practising calm scenarios in your mind — visualisation — can genuinely change how your body responds to labour.

When you regularly practise relaxation techniques during pregnancy, you are essentially training a pathway in your brain. By the time labour arrives, reaching that state of calm becomes much quicker and easier — because you've been there hundreds of times before.

This is called neuroplasticity — the brain's ability to form new habits and pathways through repetition. It works whether you're learning to play guitar or learning to breathe through a contraction.

What hypnobirthing looks like in real life

Some people using hypnobirthing are very quiet and deeply inward during labour. Others are chatting between contractions, using their breathing only during each surge. Some have pools and candles and a carefully curated playlist. Others are in a fluorescent hospital room and still use every technique they've practised.

Hypnobirthing is not an aesthetic. It's a toolkit. You use what helps, when it helps, in whatever birth environment you find yourself in.

It works just as well for caesarean births, induced labours, and complicated pregnancies as it does for straightforward ones. In fact, it may be even more valuable when things don't go to plan — because the tools help you stay calm and think clearly when you most need to.

What to expect from this course

Over the next six sessions you'll learn:

- The breathing techniques that form the foundation of hypnobirthing
- How to use light touch massage to promote relaxation and oxytocin
- What actually happens in each stage of labour — and how your tools apply
- How to prepare your birth environment and write a birth preferences document
- What to expect in the immediate postnatal period and the first two weeks

Each session ends with a workbook to complete and a homework task to do before the next session. The homework matters — these techniques only work if you practise them. A few minutes each day is more powerful than an hour once a week.

SESSION RECAP

- Hypnobirthing is evidence-based preparation for birth — not a guarantee, not woo, not only for certain people
- The Fear-Tension-Pain cycle (Grantly Dick-Read, 1933) explains why fear makes labour harder — and why addressing it matters
- Adrenaline = tension and slowed labour. Oxytocin = effective contractions and

progress

- Regular practice trains your brain to reach calm states quickly — this is neuroplasticity, not magic
- Hypnobirthing works in any birth scenario, with any birth partner, in any environment

📓 Session 1 Workbook

What were your beliefs about birth before starting this course? Where did they come from?

What are you most hoping hypnobirthing will help with?

What does your ideal birth experience look and feel like? (There are no right answers — be honest.)

What does your partner want to get from this course?

✏️ BEFORE THE NEXT SESSION — YOUR HOMEWORK

This week, try to do the following before Session 2:

1. Listen to a hypnobirthing relaxation recording each night before bed — there are free ones on YouTube and Spotify. Search 'hypnobirthing relaxation for pregnancy'. Notice how your body feels different afterwards.
2. Talk to your birth partner about what birth means to each of you. Share your answers from the workbook above.
3. Write down three things that currently make you feel calm — a place, a person, a smell, a piece of music. You'll use these later when we build your relaxation toolkit.

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Session 2 — Breathing Techniques

Best completed around 29–31 weeks

Why breathing is the foundation

Of all the hypnobirthing tools, breathing is the one you'll use the most — and the one that requires the least equipment. You can use it at 3am when a contraction wakes you up. You can use it in the back of a taxi on the way to hospital. You can use it with an epidural, in a theatre, or in a birth pool.

Controlled breathing does three things during labour. First, it ensures your baby receives a steady supply of oxygen throughout each contraction. Second, it activates your parasympathetic nervous system — the 'rest and digest' state that counteracts adrenaline and keeps you calm. Third, it gives you something to do, which keeps your thinking brain occupied and stops it catastrophising.

You're going to learn two main breathing techniques. They sound simple. They are simple. But they need to be practised until they feel completely automatic — because in active labour, you don't have the headspace to remember instructions. You just want to breathe.

The Up Breath — for contractions

This is the breath you'll use during a contraction (sometimes called a surge in hypnobirthing). The idea is that your uterus is working upward during a contraction — the muscles are drawing up and shortening. Your breath mirrors that direction.

How to do it:

- Breathe in slowly through your nose for a count of 4
- Let your belly rise as you breathe in — not your shoulders
- Breathe out slowly through your mouth for a count of 6 to 8
- Imagine breathing up and out — like you're gently blowing out a candle a metre in front of you
- Keep your jaw loose and your shoulders dropped
- Repeat for the full length of the contraction

The key is that the out-breath is longer than the in-breath. This is what activates the vagus nerve and stimulates the parasympathetic nervous system — slowing your heart rate, lowering blood pressure, and reducing the sensation of pain.

> **The science bit**
>
> *Research from the University of Pisa (2006) found that slow breathing at around 6 breaths per minute significantly increases the activity of the baroreflex — the body's blood pressure regulation mechanism — which is directly linked to reduced heart rate and greater feelings of calm. This is the physiological basis for the Up Breath.*

The Down Breath — for the second stage

The Down Breath is used during the second stage of labour, when your body is expelling

your baby. Rather than forced pushing — which most of us imagine from TV dramas — hypnobirthing encourages 'breathing your baby down'. This is gentler, more sustainable, and reduces the risk of perineal tearing.

How to do it:

- Take a full breath in through your nose
- Drop your chin slightly to your chest
- Direct your breath downward — imagine breathing down into your pelvis and perineum
- Use the out-breath to gently bear down — not a forceful push, more like a sustained, controlled bearing down
- Think of it like blowing up a balloon very slowly — steady pressure, not a burst
- Between surges, let your body completely relax and rest

Some people describe it as breathing their baby out rather than pushing them out. Research from the Cochrane library confirms that directed pushing (the purple pushing often seen on TV) is not more effective than spontaneous, maternal-led bearing down — and may cause more perineal trauma. Breathing your baby down is both gentler and evidence-based.

Practising together — a note for partners

This is where partners become genuinely useful. Your job is not to demonstrate the breathing — it is to model it. If you breathe slowly and calmly, your partner's nervous system will naturally synchronise with yours. This is called co-regulation — it is a real physiological phenomenon, and it is one of the most powerful things you can do in a labour room.

Practise the Up Breath together every evening. Set a timer for 60 seconds (roughly the length of an active labour contraction) and breathe together. Your partner leads; you follow. Do this enough times and it will become automatic for both of you.

> **Practice note**
>
> *The Up Breath should be practised every single day from now until birth — ideally for 5 minutes each evening before bed. This is not optional homework. This is the single most important thing in this course. Do it daily and it will be there for you when you need it most.*

Your birth breath — finding your own version

Some people do better with a longer out-breath — 4 in, 8 out. Some prefer counting silently; others prefer a word ('in' on the inhale, 'calm' on the exhale). Some like to visualise something during the breath — a wave, a door opening, a colour.

There is no wrong way to do this. Experiment during the next few weeks and find what works for you. The version that feels most natural when you're practising is the version that will be there in labour.

Breathing for anxiety — not just for labour

The breathing techniques in this course are not just for birth. They work for any situation that triggers anxiety — a difficult appointment, a scan you're nervous about, lying awake at

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3am unable to switch your brain off.

The 4-7-8 breath is a useful addition to your toolkit: inhale for 4, hold for 7, exhale for 8. This is a particularly effective technique for getting back to sleep or calming a racing heart. It takes about 60 seconds to feel the effect.

SESSION RECAP

- The Up Breath (in for 4, out for 6-8) is used during contractions — it activates the parasympathetic nervous system and reduces pain perception
- The Down Breath is used in the second stage — breathing the baby down rather than forced pushing reduces perineal trauma and is evidence-supported
- The longer the out-breath relative to the in-breath, the greater the calming effect on the nervous system
- Co-regulation: when your partner breathes calmly, your body follows. This is physiology, not psychology
- Daily practice is what makes these techniques work — 5 minutes a day from now until birth

📓 **Session 2 Workbook**

Practise the Up Breath right now for 60 seconds. Write down how your body felt before and after.

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What do you naturally do when you're anxious? (Hold your breath? Breathe shallowly?) Notice this — awareness is the first step to changing it.

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Partner: what word, image, or sound could you offer during contractions to anchor your partner's breathing? (e.g. 'breathe', 'you're doing brilliantly', a hand on the shoulder)

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✏️ **BEFORE THE NEXT SESSION — YOUR HOMEWORK**

Before Session 3:

4. Practise the Up Breath for 5 minutes every evening — set a reminder on your phone. Do it together if your partner is available. Use it any time you feel anxious or stressed between now and birth.

5. Build your birth playlist. Choose 10–15 songs that make you feel calm, safe, or powerful. Create it as an actual playlist you can access in labour. Think about: a slow opening track for the latent phase, something rhythmic for active labour, something soft for rest periods. Music activates the limbic system — it genuinely changes your brain state.
6. Notice your breathing in daily life this week. When do you hold your breath? When do you breathe well? No judgement — just awareness.

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Session 3 — Light Touch Massage, Perineal Massage & the Oxytocin Connection

Best completed around 31–33 weeks

Why touch matters in labour

Touch is one of the most powerful tools available to you during labour — and one of the most underused. Gentle, reassuring touch from a trusted person releases oxytocin, reduces cortisol, and activates the same calming pathways as breathing. This is not an opinion; it is well-documented physiology.

Studies from the Touch Research Institute at the University of Miami have shown that massage during labour significantly reduces pain perception, anxiety, and the need for pharmacological pain relief. A Cochrane review of continuous labour support found that the presence of a supportive companion who provides physical contact is associated with shorter labours and better outcomes.

Touch also communicates something that words cannot: you are safe, you are not alone, and I am here. In a state of heightened vulnerability, that message matters enormously.

Light touch massage — the technique

Light touch massage is not deep tissue work. It is the opposite — feather-light strokes that activate the C-tactile afferents in the skin. These are nerve fibres specifically designed to respond to gentle, social touch. When they're activated, they send signals directly to the insula — the part of the brain associated with emotional processing and wellbeing — which triggers oxytocin release.

This is sometimes called the 'calm and connect' nervous system pathway. It works best when the touch is:

- Very light — almost ticklish
- Slow — around 3–5cm per second (roughly the speed of a gentle stroke)
- Skin to skin where possible
- Rhythmic and repetitive

Where to use it:

- The lower back and sacrum — particularly effective during back labour
- The shoulders and upper back — especially in early labour when standing or leaning forward
- The arms — slow strokes from shoulder to elbow during a contraction
- The scalp — if welcome; some people find this deeply relaxing

> **Partner note**
>
> *Practice this now. Ask your partner to close their eyes. Use three or four fingers and draw slow, light strokes down their arm from shoulder to wrist. The pressure should be just enough to be felt — no heavier. Ask for feedback. Most people are surprised at how quickly this produces a feeling of calm. You'll use this in labour without even thinking*

about it.

Back massage for labour

Counter-pressure on the sacrum — the large bone at the base of the spine — is one of the most effective techniques for managing back labour (when your baby is in a posterior position and the back of their head is pressing against your spine).

To apply counter-pressure: make a fist or use the heel of your hand and press firmly and steadily against the sacrum during a contraction. This is not light touch — it is firm, static pressure. Many people describe it as providing immediate and significant relief.

Experiment with both techniques — light touch on the rest of the back, counter-pressure on the sacrum — and find what works best. Preferences often change during labour, so partners should be prepared to switch between the two.

Perineal massage — what it is and why it matters

Perineal massage is a technique for preparing the perineum — the area between the vagina and the anus — for the stretching that occurs during the second stage of labour. It's recommended from around 34–35 weeks and takes about 3–5 minutes per session.

Research is clear on this one. A Cochrane review (Beckmann & Stock, 2013) found that regular perineal massage from 35 weeks significantly reduces the risk of perineal trauma requiring sutures, particularly for first-time parents. It also reduces the likelihood of an episiotomy and is associated with less postnatal perineal pain.

How to do perineal massage:

- Start from around 34–35 weeks, 3–4 times per week
- Use a natural oil — sweet almond oil, coconut oil, or a dedicated perineal massage oil
- Wash your hands and find a comfortable position — lying back with knees bent, or one foot up on the side of the bath
- Insert both thumbs or index fingers about 3–4cm inside the vagina
- Apply gentle downward and outward pressure toward the rectum — you should feel a stretching sensation, not pain
- Hold for 30–60 seconds, then slowly move in a U-shape from side to side
- The sensation should be a firm stretch, not burning or sharp pain
- Total time: 3–5 minutes per session

This can feel awkward at first — that is entirely normal. It gets easier and more comfortable with practice, which is part of the point. Your perineum is literally learning that stretching is safe.

Research corner

Beckmann & Stock (2013) Cochrane review: women who practised perineal massage from 35 weeks were significantly less likely to have an episiotomy or tear requiring sutures. The benefit was greatest for first-time mothers. This technique has the strongest evidence base of almost anything in the hypnobirthing toolkit.

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Building your touch toolkit together

Before you reach active labour, you and your partner need to have genuinely practised these techniques. Labour is not the time to learn. Spend 10–15 minutes this week trying everything: light touch strokes, counter-pressure on the sacrum, and experimenting with positions while you practise.

Find out what feels good now, so neither of you is guessing later.

SESSION RECAP

- Touch activates C-tactile afferents, which trigger oxytocin release — this is why gentle touch is a genuine pain management tool in labour
- Light touch massage works best when it is slow, light, rhythmic, and skin-to-skin
- Counter-pressure on the sacrum provides immediate relief for back labour — firm, static pressure during contractions
- Perineal massage from 35 weeks (3–4 times a week) reduces the risk of tearing and episiotomy — Cochrane-reviewed evidence
- Practise everything now — labour is not the time to learn a new technique

📓 Session 3 Workbook

Partner: practise light touch massage on your partner's arm right now. Ask: how does the pressure feel? Too light? Too heavy? Write down what they preferred.

Where on your body do you most like to be touched when you need comfort? Tell your partner.

How do you feel about perineal massage? Does anything feel like a barrier to trying it? Write it down — naming a concern is often enough to dissolve it.

✏️ BEFORE THE NEXT SESSION — YOUR HOMEWORK

Before Session 4:

7. Begin perineal massage this week if you are at 34+ weeks. Aim for 3 times this week. If you're earlier, add it to your calendar from 34 weeks.
8. Partner: practise light touch massage on your partner's back for 5 minutes —

once this week, ideally during one of your evening breathing practices.

9. Listen to a guided relaxation recording while your partner practises light touch. Notice whether the combination feels different to breathing alone.
10. Finalise your birth playlist — test it and adjust it. Share it with your partner and explain why you chose each song.

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Session 4 — The Stages of Labour

Best completed around 33–35 weeks

Knowing what's coming makes it less scary

One of the most powerful things you can do before birth is understand exactly what is going to happen. Fear of the unknown is one of the biggest contributors to the Fear-Tension-Pain cycle. When you know what a contraction is doing, what the different stages involve, and roughly what to expect — the whole thing becomes far less frightening.

We're going to walk through each stage of labour and show you exactly where your hypnobirthing tools apply. Think of this as the map for your journey.

The Latent Phase — early labour

The latent phase is the opening act. Your cervix is softening, thinning (effacing), and beginning to open — from 0 to 4cm dilation. Contractions start irregular and spaced apart, typically 10–20 minutes, and last around 20–40 seconds. This phase can last anywhere from a few hours to a few days, especially for first-time parents.

This is the part nobody tells you about — and it's often the hardest psychologically, because it can feel like a very long time before anything seems to be 'happening'.

Your hypnobirthing tools at this stage:

- Up Breath for contractions — use it from the very first surge, even if they feel mild. You're training your nervous system.
- Normal activity between contractions — sleep if it's night, eat and drink, walk slowly, watch something easy on TV. Don't sit and stare at a clock.
- Light touch massage — ask your partner to start during contractions if you want grounding
- Relaxation recordings — brilliant for rest periods, especially if this phase runs overnight
- Stay at home — for most people, early labour is most comfortably managed at home. Contact your midwife if you have any concerns.

> **Tip**
>
> *Have a bag of snacks ready, a good playlist, and something easy to watch. The latent phase is a marathon, not a sprint. Rest, eat, drink, and let your body do its quiet work. The contractions that feel manageable now are doing exactly what they need to do.*

First Stage — active labour

Active labour begins at around 4cm dilation and runs to full dilation at 10cm. Contractions are now longer (45–60 seconds), stronger, and closer together (typically 3–5 minutes apart). This is when most people head to their birth setting.

This is the stage where hypnobirthing really earns its place. Contractions at this stage are intense — they need to be, because they're doing significant work. But they are also finite. Each one has a beginning, a middle, and an end. There is always a break afterwards.

Your hypnobirthing tools at this stage:

- Up Breath — your anchor for every contraction. In through the nose, long out through the mouth. Focus only on the breath, not what comes next.
- Birth positions — stay upright and mobile if you can. Kneeling, all fours, leaning on a birth ball, walking between contractions. Gravity helps. Lying on your back is usually the most uncomfortable position in active labour.
- Light touch massage and counter-pressure — partner: now is when this really matters. Be present, consistent, and calm.
- Water — if a birth pool or deep bath is available, get in. Warm water provides significant pain relief and promotes oxytocin.
- Your birth environment — dim lights, familiar scents, your playlist. Signal to your nervous system that you are safe.
- Co-regulation — if your partner breathes calmly and speaks softly, your nervous system follows. This is the most important thing a partner does.

> **For partners**
>
> *In active labour, your voice, your presence, and your calm are more important than anything else. Talk less. Touch more. Breathe. Don't tell her how dilated she is or how far she has to go. Focus entirely on the current contraction. Just this one. That's all that needs to exist right now.*

Transition — the bridge between first and second stage

Transition is the shortest and most intense phase of labour. It happens as the cervix finishes dilating and the body shifts from contracting to expelling. Contractions are very close together, sometimes with almost no break. Many people feel overwhelmed, shaky, nauseous, or suddenly convinced they cannot continue.

Here is the single most important thing to know about transition: it means you are almost there. The feelings of overwhelm are a physiological signal that birth is close. If you or your partner are thinking 'I can't do this' — that thought is a sign you are at the end of the hardest part, not the beginning of it.

Your hypnobirthing tools at this stage:

- Breathe. Just breathe. The Up Breath — in through the nose, long out through the mouth. That's all.
- Partner: do not leave the room. Be physical — hand, shoulder, presence. Say 'you're so close' and mean it.
- Don't push until your body is ready — your midwife will guide you. If you feel the urge before full dilation is confirmed, pant or breathe through the surges.

Second Stage — birth

The second stage begins at full dilation and ends with the birth of your baby. It can last from a few minutes to a few hours (longer for first-time parents). Your uterus is now working differently — the sensation shifts from contracting to an overwhelming urge to bear down.

Your hypnobirthing tools at this stage:

- Down Breath — breathe your baby down with each surge. Steady, controlled bearing down rather than held-breath forced pushing.

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- Trust your body's urges — your body knows how to do this. The bearing-down reflex is powerful and instinctive.
- Rest completely between surges — let your uterus do the work. Conserve your energy.
- Stay in positions that open the pelvis — squatting, kneeling, or side-lying all increase pelvic dimensions significantly more than lying on your back.
- Warm compress on the perineum — if your midwife offers this during crowning, accept it. It significantly reduces the sensation of burning and reduces tearing risk.

Third Stage — delivering the placenta

The third stage is the delivery of the placenta after your baby is born. You will be offered an injection of synthetic oxytocin (Syntocinon) to help your uterus contract and the placenta deliver — this is called active management of the third stage and is recommended by the RCOG as it reduces the risk of heavy bleeding.

If you choose physiological management (no injection), the placenta is delivered by your own natural oxytocin — which is maximised by skin-to-skin with your baby immediately after birth. Keep your environment calm, baby on chest, and let nature take its course.

Either way: your hypnobirthing tools remain relevant here. Stay calm, breathe, and enjoy meeting your baby. The hard work is done.

SESSION RECAP

- Latent phase (0-4cm): use breathing for every contraction, rest between them, stay home, eat and drink
- Active labour (4-10cm): stay upright and mobile, use Up Breath consistently, use touch and water, create a calm environment
- Transition: the most intense but briefest phase — 'I can't do this' means you're almost there
- Second stage: Down Breath to breathe your baby down, trust the bearing-down reflex, rest between surges
- Third stage: discuss active vs physiological management with your midwife, skin-to-skin supports oxytocin for both

📓 Session 4 Workbook

Which stage of labour feels most intimidating to you right now? Write down what specifically worries you about it.

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Partner: what is your plan for each stage? What will you do during a contraction? Between contractions? Write a sentence for each stage.

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What positions do you want to try in labour? Have you tried any of them at home?

✏️ **BEFORE THE NEXT SESSION — YOUR HOMEWORK**

Before Session 5:

11. Do a 'labour rehearsal' together. One of you sets a timer for 60 seconds. The other practises a contraction: breathing through it, using a position, having their partner provide touch. Do this 5 times with a 3-minute rest between each. This feels silly. Do it anyway — it works.
12. Watch a positive birth video together — search for 'hypnobirthing birth video' on YouTube. Watch the breathing, the partner support, and the atmosphere rather than the medical details.
13. If you haven't started perineal massage yet, start this week. Aim for 3 sessions before Session 5.
14. Write a first draft of your birth preferences — a list of your wishes for labour, birth, and the immediate postnatal period. We'll refine it in Session 5.

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Session 5 — Your Birth Environment, Expectations & Birth Preferences

Best completed around 36–37 weeks

Environment matters — a lot

Where you give birth, and how that environment feels, has a measurable effect on how your labour progresses. This is not about aesthetics — it is about neuroscience. A mammal in labour needs to feel safe, private, and undisturbed. When those conditions are present, oxytocin flows freely and labour progresses. When they're absent, adrenaline rises and labour stalls.

Professor John Kennell's research on continuous labour support found that the presence of a calm, known companion in the birth room was associated with shorter labours, fewer interventions, and higher maternal satisfaction. Your environment and the people in it are clinical variables — not optional extras.

Your birth environment checklist

Whatever your birth setting — home, birth centre, or hospital — there are things you can control and things you can't. Focus your energy on what you can control.

Lighting:

- Dim the lights if you can — bright overhead fluorescent lights activate the sympathetic nervous system
- Bring a battery-powered fairy light or LED tea lights if your unit allows
- A sleep mask can block out light if you can't dim the room

Sound:

- Your birth playlist — have it downloaded, not streaming, in case of poor signal
- A small portable speaker if allowed
- Let your midwife know you're using hypnobirthing so they know to keep their voice calm and the room quiet

Scent:

- Lavender essential oil on a flannel or in a diffuser is associated with reduced anxiety in labour — supported by several small clinical trials
- Clary sage is traditionally associated with labour onset — do not use before 38 weeks
- A pillow from home smells like safety

Temperature:

- Warm for most of labour (promotes relaxation and oxytocin)
- A cool flannel on the forehead and neck during transition provides significant comfort

People:

- Only have people in the room who make you feel safe. Every person who makes you feel observed or judged raises your adrenaline.

- Brief your birth partner in advance — calm energy, minimal talking during contractions, consistent presence
- It is completely reasonable to ask medical staff to knock before entering and to speak quietly

Birth preferences — not a birth plan

We call it birth preferences rather than a birth plan for good reason. Labour doesn't always follow a plan. Having a rigid plan can make it harder to adapt when things change — and things often change. But having clear preferences means your care team understand what matters most to you, which helps them support you better in every scenario.

A good birth preferences document is:

- Short — one page if possible. Busy midwives will read one page. They may not read four.
- Positive — 'I would like' rather than 'I do not want'
- Flexible — include a section for 'if I need a caesarean' or 'if induction is recommended'
- Shared in advance — give a copy to your midwife at your 36-week appointment

What to include:

- Birth environment preferences (lighting, music, who is present)
- Pain relief preferences — what you'd like to try first, what you're open to
- Positions you'd like to try
- Monitoring preferences — mobile monitoring if available
- Second stage preferences — breathing down, positions, warm compress
- Third stage — active or physiological management
- Immediate postnatal — skin-to-skin, delayed cord clamping, vitamin K preferences
- Feeding intentions — breastfeeding, formula, combination
- If caesarean: skin-to-skin in theatre, delayed cord clamping, partner present

> **Remember**
> *Your birth preferences are a communication tool, not a contract. Your midwife team want to support your wishes. Writing them down helps them do that, especially if there is a handover of care during your labour. Discuss them at your next appointment.*

Managing expectations

Hypnobirthing does not promise a painless birth, a short birth, or a birth that goes to plan. What it promises is that you will have tools to manage whatever happens — and that you will feel more in control of your experience, whatever it looks like.

Some things to hold gently:

- Labour hurts for most people. The goal is not to eliminate that sensation but to change your relationship to it. A contraction is your body working powerfully — not your body failing.
- Plans change. A planned home birth may become a hospital birth. A planned vaginal birth may become a caesarean. None of these outcomes make you less prepared, less courageous, or less of a success.

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- Your birth partner will not do everything perfectly. Neither will you. Good enough, done with love and presence, is more than enough.
- The birth is one day. The parenting lasts a lifetime. Keep perspective.

SESSION RECAP

- Your birth environment is a clinical variable — light, sound, scent, temperature, and people all affect oxytocin and adrenaline levels
- Birth preferences (not a birth plan) should be one page, positive, flexible, and shared with your midwife at 36 weeks
- Hypnobirthing doesn't guarantee a pain-free or complication-free birth — it gives you tools for every scenario
- Every birth is a success. The goal is not a specific outcome — it is feeling informed, respected, and supported throughout

📓 **Session 5 Workbook**

What three things matter most to you about your birth experience? (Not outcomes — experiences. Feeling heard? Being able to move freely?)

_____

_____

Write a one-paragraph 'if I need a caesarean' section for your birth preferences. What would you want in that scenario?

_____

_____

Partner: how will you prepare yourself emotionally for birth? What do you need in order to stay calm? Write it down.

_____

_____

✏️ **BEFORE THE NEXT SESSION — YOUR HOMEWORK**

Before Session 6:

15. Finalise your birth preferences document — print two copies and put one in your hospital bag, one in your maternity notes.
16. Visit your birth setting if you haven't already — many units offer tours. Knowing what the room looks like removes one unknown.
17. Have a 'birth partner briefing' — an honest conversation where your partner tells you what they're worried about and you tell them the same.
18. Pack your hospital bag this week.

19. Continue daily Up Breath practice, perineal massage, and your relaxation recording.

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Session 6 — The Baby Moon & the Fourth Trimester

Best completed around 38–39 weeks

The part nobody prepares you for

Almost everything written about birth focuses on the birth itself. The days and weeks that follow — what many people call the fourth trimester or the baby moon — receive far less attention, despite being one of the most physically and emotionally intense periods of most people's lives.

The term 'baby moon' describes the idea of a cocoon of calm, closeness, and recovery in the first weeks after birth. Some cultures protect this time fiercely — from Ayurvedic traditions in India to the Chinese practice of 'sitting the month'. Modern Western culture tends to expect new parents to bounce back immediately. This expectation is not only unrealistic; it is actively harmful.

You have just run the equivalent of a physiological marathon. And your baby has just made the biggest transition a human being will ever make. You both need time.

What happens to your body in the first two weeks

In the immediate aftermath of birth, your body undergoes some of the most dramatic hormonal changes of your life. Oestrogen and progesterone drop precipitously. Oxytocin surges during skin-to-skin and breastfeeding. Prolactin rises. Cortisol fluctuates with sleep deprivation.

Normal postnatal experiences in the first two weeks:

- Lochia (postnatal bleeding) for up to 6 weeks — starting red and heavy, gradually lightening
- Afterpains — strong cramping as the uterus contracts back to pre-pregnancy size, particularly during breastfeeding
- Breast engorgement around days 3–5 as mature milk comes in
- The baby blues — tearfulness and emotional lability peaking around day 3–5, usually resolving within two weeks
- Perineal soreness, swelling, and healing — ice packs and warm baths help
- Extreme fatigue — you are not sleeping and your body is recovering from a major event

Continuing your hypnobirthing practice postnatally

The tools you've learned in this course don't expire at birth. They remain genuinely useful in the weeks that follow.

Breathing:

The Up Breath — your long slow out-breath — is your most useful tool for the immediate postnatal period. Use it:

- During painful afterpains, particularly during breastfeeding
- During postnatal checks when you may have stitches examined or the uterus palpated
- During the moments of overwhelm that will come — and are entirely normal

- To get back to sleep after a night feed

Touch:

Skin-to-skin contact with your baby in the first hours and days does everything your hypnobirthing practice was working toward: it releases oxytocin in both of you simultaneously, stabilises your baby's temperature and blood glucose, supports breastfeeding, and lays the foundations of attachment.

The golden hour — the first 60 minutes after birth — is the most powerful window for skin-to-skin. If possible, delay any non-urgent newborn procedures until after this hour has passed.

Relaxation:

Rest when the baby rests — even if you can't sleep, lying quietly with your breathing practice is restorative. Your nervous system is in a state of heightened arousal after birth; regular rest periods help regulate it.

Safe sleep — what your baby needs

Safe sleep is one of the most important things you can do to protect your newborn. The Lullaby Trust safe sleep guidance:

- Always place your baby on their back to sleep
- In a clear sleep space — no loose bedding, pillows, bumpers, or soft toys
- In the same room as you for the first 6 months (but not in your bed — unless following safe co-sleeping guidance)
- In a room temperature of 16–20°C
- Never on a sofa or armchair with a sleeping adult
- Smoke-free environment always

These guidelines reduce the risk of sudden infant death syndrome (SIDS) by up to 50%. They are not optional.

Your emotional wellbeing

The baby blues are a normal, hormone-driven emotional response to birth. Tearfulness, overwhelm, and a sense of surreal disconnection in the first week are almost universal. They are not postnatal depression and they don't mean anything is wrong.

Postnatal depression (PND) is different — it is persistent (lasting more than two weeks), characterised by low mood most days, an inability to enjoy things, and sometimes difficulty bonding with your baby. PND affects approximately 1 in 10 new mothers and 1 in 25 new fathers. It is not a character failing. It is a clinical condition with excellent treatment options. If any of this sounds familiar — tell your midwife or health visitor.

> **A note on identity**
>
> *Becoming a parent involves a shift in identity that is as significant as any developmental stage — sometimes called matrescence. Your sense of who you are, what you value, and how you relate to the world changes. This can feel disorienting even when it is also joyful. It is normal. It takes time. Be patient with the version of yourself that is still emerging.*

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The baby moon in practice

Say yes to help, no to unnecessary visitors:

Decide in advance with your partner how you'll manage visitors. Ideally, the first week is for the three of you only. Beyond that, only people who bring food, do laundry, or leave when asked. This is not anti-social — it is sensible self-protection during one of the most vulnerable transitions of your life.

Outsource everything that isn't feeding, resting, or holding your baby:

Cleaning, admin, cooking, social media updates — all of it can wait or be done by someone else. Your one job right now is recovery and meeting your baby.

SESSION RECAP

- The fourth trimester (the first 12 weeks after birth) is a period of recovery, hormonal change, and enormous adjustment for both parent and baby
- The baby moon is not a luxury — it is physiologically protective. Protect that time.
- The Up Breath remains your most useful postnatal tool — for afterpains, procedures, overwhelming moments, and sleep
- Skin-to-skin contact and the golden hour are the most evidence-based immediate postnatal interventions available
- Baby blues (first week, hormone-driven) are normal. PND (persistent, starting after week 1) needs professional support — ask for it without hesitation
- Safe sleep guidelines reduce SIDS risk by up to 50% — follow them every time, from day one

📓 Session 6 Workbook

What does your ideal baby moon look like? Who would you want around? What would a typical day look like?

_____

_____

_____

Write your visitor policy — who, when, for how long, and what they're expected to bring or do.

_____

_____

Partner: what are you most worried about in the first two weeks? Write it down. Then write one thing you can do to prepare for it.

_____

_____

What support do you have around feeding? Do you know how to access a

breastfeeding counsellor if you need one?

_____

_____

✏️ BEFORE THE NEXT SESSION — YOUR HOMEWORK

Your final homework — and the most important:

20. Continue your Up Breath practice every single day until birth. This is the one thing that makes the biggest difference.
21. Continue perineal massage 3–4 times a week until 38 weeks.
22. Read your birth preferences document out loud to your partner. Does it still feel right? Adjust anything that has changed.
23. Do one final 'labour rehearsal' together — breathing through 5 contractions, with massage, with your playlist.
24. Tell someone you love that you're ready. Say it out loud. You are.

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Quick Reference — Your Hypnobirthing Toolkit

Breathing at a glance

Technique	How	When to use
Up Breath	In through nose ×4, out through mouth ×6–8	All contractions in latent and active labour
Down Breath	Full breath in, chin down, breathe down and out steadily	Second stage — bearing down with each surge
4-7-8 Breath	In ×4, hold ×7, out ×8	Anxiety, insomnia, any time you need to calm quickly

Stages of labour at a glance

Stage	What's happening	Your tools	Partner's role
Latent phase	0–4cm. Irregular contractions.	Up Breath, relaxation, stay home, eat and drink	Calm presence, light touch, food, timing
Active labour	4–10cm. Regular, strong contractions.	Up Breath, positions, water, calm environment	Touch, counter-pressure, breathe with you
Transition	Final dilation. Very intense, brief.	Up Breath only. Just breathe.	Do not leave. 'You're almost there.'
2nd stage	Birth. Bearing-down urge.	Down Breath, positions, trust your body	Encourage, support positions, cool flannel
3rd stage	Placenta delivery.	Breathe, skin-to-skin, oxytocin	Stay with mum and baby. Quiet and calm.
Baby moon	First 2 weeks.	Rest, skin-to-skin, Up Breath for afterpains	Protect the space. Do everything else.

Perineal massage — quick guide

From 34–35 weeks, 3–4 times per week

Use: clean hands, natural oil (almond, coconut, or perineal preparation oil)

Position: lying back with knees bent, or one foot elevated

Method: insert thumbs 3–4cm, apply downward pressure, hold 30–60 seconds, sweep slowly side to side

Feel: firm stretch — not burning, not sharp pain

Time: 3–5 minutes per session

Evidence: Cochrane review (Beckmann & Stock, 2013) — significantly reduces tearing and episiotomy risk, especially for first-time parents

References & further reading

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